

OREGON STATE HOSPITAL

POLICY

SECTION 7: Patient Rights **POLICY: 7.015**

SUBJECT: Patient Rights Restrictions
Requirements

POINT PERSON: Director of Ombuds and Family
Services

APPROVED: Sara Walker, MD  **DATE: SEPTEMBER 25, 2024**
Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of
existing policy
☒ Reaffirmation of existing policy ☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) is committed to providing patient-centered care in a safe, humane environment where a patient's rights are protected. This policy outlines if and when rights may need to be restricted, and responsibilities of staff in these situations.
- B. This policy applies to all staff.

II. POLICY

- A. A patient's rights may only be restricted for safety, security, or other clinically necessary reasons. A patient's rights cannot be restricted as punitive measures, nor may their rights be considered privileges to be earned. Restrictions to their rights may only be implemented as delineated in this policy and within boundaries established by federal or state regulations for good cause, meaning sufficient reason to take action. In this policy, good cause for restricting a patient's rights includes determining:
1. the exercise of the specific right would be injurious to the patient or others;
 2. there is evidence that the specific right, if exercised, would seriously infringe on rights of others;
 3. the patient is engaging in illegal behaviors; or
 4. a restriction is considered necessary for the safety and security of OSH.

- B. Staff will use the least restrictive means of addressing the behavior that led to considering a restriction. Before implementing a restriction to a patient's right, staff must offer or try alternative interventions.
- C. Any restriction to a patient's right must be:
 - 1. clinically justified for good cause,
 - 2. ordered by a psychiatrist or psychiatric mental health nurse practitioner (PMHNP),
 - 3. explained to the patient within 24 hours of the restriction, and
 - 4. documented in progress notes.
- D. The reason for a restriction must be related to the right being restricted. A restriction must be lifted as soon as possible and may not be in effect longer than necessary for clinical or safety/security reasons.
- E. A member of the Interdisciplinary Treatment Team (IDT) must promptly notify a patient verbally and in writing whenever a right is restricted and when the IDT reviews and extends the restriction. Patient right restriction notifications identified in Attachment A must be documented on a "Patient Notification Rights Restriction" form 76126 MR-1. Exceptions to written notification include enhanced supervision and seclusion or restraint.
- F. When a restriction is established, the IDT must develop an individualized plan to reinstate the patient's rights and document it in their treatment care plan (TCP).
- G. Documentation of any restriction must include:
 - 1. the clinical justification;
 - 2. assessment of the need for the restriction;
 - 3. alternative interventions attempted;
 - 4. the date and time the restriction was established;
 - 5. the date the restriction will be reviewed (no more than 30 days after the restriction was implemented);
 - 6. the reason for continuing the restriction, if applicable;
 - 7. a plan for reinstating the patient's right;
 - 8. when indicated, a completed, approved "Restriction of Patient Rights Approval and Notification" form 76125 MR-1 ; and
 - 9. when the patient was notified verbally and in writing of the restriction.
- H. For a restriction to be established or extended, staff must promptly obtain appropriate approvals from the OSH Ombuds and Family Services Office (OFS), Program Director, and other appropriate persons per the "Restriction of Patient Rights Approval and Notification" form.

NOTE: Exceptions to this process include enhanced supervision and seclusion or restraint.

1. The IDT must review and reconsider the restriction at least monthly while it is in effect. If the IDT extends the restriction, they must promptly email restriction extension notifications at 30 days and 60 days after the restriction started. Each notification must include the justification for the restriction or restriction extension and a revised restriction plan.
2. If the IDT determines that the restriction should continue past 90 days, they must obtain approval from the Program Executive Team and OFS before continuing the restriction. When the "Restriction of Patient Rights Approval and Notification" form is required, the IDT must submit an updated form, the justification for the restriction, and a revised restriction plan at 90 days afterwards and again at every 30-day extension request.
- I. In an emergency where there is significant risk of harm to the patient or others, a registered nurse (RN) may establish an emergency temporary restriction if it is:
 1. clinically justified for good cause;
 2. ordered by a psychiatrist/PMHNP;
 3. promptly explained to the patient verbally and in writing; and
 4. documented, including:
 - a. the clinical justification,
 - b. assessment of the need for the restriction,
 - c. alternative interventions attempted,
 - d. the date and time the restriction was established, and
 - e. the date the IDT will review the restriction (the next business day).
- J. An emergency temporary restriction must be lifted as soon as possible and may not be in effect longer than clinically necessary.
 1. The emergency temporary restriction must be promptly reviewed by the patient's IDT the next business day. In no situation may it be in effect beyond the IDT's review.
 2. The IDT must complete requirements listed in this policy to extend the restriction. Before an emergency temporary restriction listed on the "Restriction of Patient Rights Approval and Notification" form can continue, the IDT must promptly submit a request to OFS and the Program Director as directed by this policy to develop a plan and obtain their approval.
 3. See applicable policies for enhanced supervision and seclusion or restraint processes.

- K. In certain circumstances the Superintendent, Administrator on Duty, or Chief Medical Officer may establish a temporary blanket restriction for all patients for medical or other safety reasons with good cause.
 - 1. Unit staff must verbally inform patients of the reason for the blanket restriction and when the restriction will end. They must post the written information in an area where patients may view it (e.g., “It is over 90 degrees outside, so we cannot go to the quads until it cools off”).
 - 2. In circumstances where a blanket restriction is implemented for medical or safety reasons, a psychiatrist/PMHNP order is not required for each patient. When in effect, IDTs are not required to document individual plans to reinstate patient rights in TCPs or submit “Restriction of Patient Rights Approval and Notification” forms.
- L. Staff, including OSH leadership, will involve or consult with OFS on all patient rights issues, including clarification and interpretation of patient rights, and before implementing related process or policy changes.
- M. If a patient right authorized by federal or state law is restricted, OFS will inform the Superintendent as needed.
- N. This policy does not apply to situations where something which is not a federal or state patient right is restricted, such as restricting a patient from going to the dining room.
- O. Only the Superintendent or their designee may make an exception to this policy for unusual or emergent circumstances.
- P. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- Q. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. “Interdisciplinary treatment team (IDT)” includes the attending psychiatrist/psychiatric mental health nurse practitioner (PMHNP), registered nurse (RN), psychologist, social worker, mental health therapist MHT or case monitor, treatment care plan specialist (TCPS), and any other clinical staff member who provides active treatment to the patient. The IDT is responsible to coordinate in planning and overseeing a patient’s care and treatment.
- B. “Staff” includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

- C. "Treatment care plan" is an individualized, person-centered care plan of treatment developed with each patient in accordance with regulations and standards. The TCP is a driver for all care provided to the patient, and is based on a patient's background, clinical assessments, case formulation, and treatment needs.

IV. ATTACHMENTS

Attachment A Patient Rights Restriction Guide

V. RELATED OSH FORMS

Restriction of Patient Rights Approval and Notification [76125 MR-1, Medical Record form]

Patient Notification Restriction of Rights Form [76126 MR-1, Medical Record form]

Patient Right Restriction Plan [76136 MR-1, Medical Record form]

VI. RELATED OSH POLICIES AND PROTOCOLS

2.006 Records Retention and Management

2.008 Privacy and Security of Patient Information

4.010 Patient Funds

6.003 Seclusion or restraints

6.010 Enhanced Supervision

6.011 Treatment Care Planning

6.020 Interpreter Language Services for Patients

6.025 Advance Directives

6.053 Education Services for Patients Under 21 Years of Age

6.056 Suicide Risk Screening and Assessment

7.001 Mail and Packages for Patients

7.003 Photographing or Recording of Patients

7.006 Patient Grievances

7.008 Patient Abuse or Mistreatment Allegation Reporting

7.009 Voter registration and Voting

7.011 Faiths, Beliefs, and Practices

7.013 Law Library, Access to Legal Materials

7.014 Medical Records Patient Requests

8.001 Video Surveillance

8.026 Visitors to Patients

8.037 Patient Property and Valuables

8.041 Patient Personal Searches

Legal Affairs Department FAQ Release of Information (ROI) [resource document]

Medical Department protocol 1.001 Informed Consent

VII. REFERENCES

42 CFR § 482.13.

42 CFR § 482.61(c)2.

42 U.S.C. § 9501 (1980).

Joint Commission Resources, Inc. (2024). The joint commission comprehensive accreditation manual for hospitals, PC 01.01.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). The joint commission comprehensive accreditation manual for hospitals, RI 01.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rules § 309-035-0175.

Oregon Administrative Rules §§ 309-102-100 — 309-118-050.

Oregon Administrative Rules §§ 333-505-0033.

Oregon Revised Statutes §§ 127.700 — 127.737.

Oregon Revised Statute § 179.485.

Oregon Revised Statute § 426.100.

Oregon Revised Statutes §§ 426.380 — 426.395.

Oregon Revised Statutes §§ 430.205 — 430.210.